

THE NUTRITION CLUB CLINIC

Diabetes & Blood Sugar Guide

Simple nutrition information for more confident blood sugar choices and everyday routines.

Personalised advice should account for your health history and medicines.

What's inside

Managing blood sugar is less about banning foods and more about the pattern on your plate, the order you eat, and how consistent your days are. This guide walks through the basics of glucose, how carbohydrate quality and quantity affect readings, what a balanced plate looks like, and how to troubleshoot the highs and lows you may see on a meter or sensor.

1. Blood sugar in plain language
2. Carbohydrates: quantity and quality
3. Building a blood-sugar-friendly plate
4. Beyond food: movement, sleep and stress
5. Monitoring and troubleshooting

How to use this guide

Read it once end to end, then pick **two** changes to work on for the next fortnight — for example, adding protein to breakfast and walking 10 minutes after dinner. Track readings alongside the change so you can see what actually works for your body.

1. Blood sugar in plain language

Glucose is the sugar your body uses for fuel. After a meal, carbohydrate is broken down into glucose and absorbed into the bloodstream. The pancreas releases insulin, a hormone that acts like a key, letting glucose move into muscle, liver and fat cells.

In **type 1 diabetes** the body makes little or no insulin, so it must be replaced. In **type 2 diabetes** cells respond poorly to insulin (insulin resistance) and the pancreas gradually struggles to keep up.

Prediabetes means glucose is above normal but not yet in the diabetes range — often the most responsive stage to nutrition and activity changes. **Gestational diabetes** appears in pregnancy and needs specialist care.

Common numbers you'll hear

Measure	What it tells you	Typical target discussed with clinicians
Fasting glucose	Level after 8+ hours without food	Often 80–130 mg/dL (4.4–7.2 mmol/L) for many adults with diabetes
2 hours after a meal	How your body handled that meal	Often under 180 mg/dL (10 mmol/L)
HbA1c	Average glucose over ~3 months	Commonly under 7% — individualised by age and health
Time in range	% of day a sensor reads in target	Often 70%+ of the day

Targets are personal

Older adults, people with heart or kidney disease, and those prone to hypos are often given looser targets on purpose. Use the numbers your own clinician sets for you rather than a figure from the internet.

2. Carbohydrates: quantity and quality

Carbohydrate raises blood glucose more than protein or fat, so it is the nutrient worth understanding best. The goal is not zero carbohydrate — it is a sensible amount of the right kind, spread through the day.

Quality: choose slower carbohydrates

- **Go-to:** whole grains (oats, barley, brown rice, whole wheat roti), beans, lentils, chickpeas, most vegetables, whole fruit, plain dairy.
- **Moderate:** white rice, white bread, potatoes, refined breakfast cereals — fine in controlled portions, better when paired with protein, fat or fibre.
- **Limit:** sugary drinks, fruit juice, sweets, pastries, sweetened tea and coffee. Liquid sugar hits the bloodstream fastest and satisfies least.

Quantity: a practical starting point

Many adults do well starting around **45–60 g of carbohydrate per main meal** and **15–20 g per snack**, then adjusting based on readings, appetite and activity. Rough 15 g portions: 1 small slice of bread, half a medium roti, ■ cup cooked rice, 1 small fruit, 1 cup milk, ½ cup beans.

Fibre is your ally

Fibre slows glucose absorption, feeds gut bacteria and improves fullness. Aim for roughly 25–30 g a day from vegetables, pulses, whole grains, nuts and seeds. Increase gradually and drink more water, or you may feel bloated.

Glycaemic index in one line

GI ranks how fast a carbohydrate food raises glucose on its own — useful, but real meals are mixtures. A high-GI food eaten with protein, fat and vegetables behaves very differently to the same food eaten alone.

3. Building a blood-sugar-friendly plate

The plate method gives you portion control without weighing anything. Use a standard 9-inch (23 cm) dinner plate.

Half the plate	A quarter	A quarter
Non-starchy vegetables — leafy greens, salad, gourds, okra, cauliflower, beans, tomatoes, peppers	Protein — chicken, fish, eggs, lentils, beans, tofu, paneer, lean meat, yoghurt	Slow carbohydrate — whole grain roti, brown rice, potato with skin, oats, corn

Add a thumb of healthy fat (olive oil, nuts, seeds, avocado) for satiety, and drink water or unsweetened tea with the meal.

Food order matters

Eating vegetables and protein **before** the starch in the same meal can noticeably blunt the post-meal glucose rise. It costs nothing to try.

A day of meals as an example

- **Breakfast:** Vegetable omelette with one slice of whole grain toast, or overnight oats with Greek yoghurt, chia and berries.
- **Lunch:** Lentil or chickpea curry, large salad, one small whole wheat roti, plain yoghurt.
- **Snack:** A handful of nuts with an apple, or hummus with cucumber and carrot.
- **Dinner:** Grilled fish or chicken, half a plate of stir-fried vegetables, ■—½ cup rice.
- **If needed:** a small protein-containing bedtime snack, especially on insulin.

Sweeteners and 'sugar-free' labels

Non-nutritive sweeteners can help you cut liquid sugar, but 'sugar-free' biscuits still contain flour and fat and will still raise glucose. Read total carbohydrate, not just sugars. Sugar alcohols like maltitol may cause wind or loose stools in larger amounts.

4. Beyond food: movement, sleep and stress

Movement

- A 10–15 minute walk after meals is one of the most effective, lowest-cost tools for post-meal glucose — muscles take up glucose without needing much insulin.
- Aim for about 150 minutes of moderate activity a week, plus resistance work (bands, weights, bodyweight) twice a week. Muscle is your largest glucose sink.
- Break up long sitting periods with 2–3 minutes of standing or walking every half hour.

Sleep

A few nights of short sleep measurably worsens insulin sensitivity and increases appetite for refined carbohydrate. Protect 7–9 hours with a consistent bedtime, a dark cool room and no screens for the last half hour.

Stress and illness

Stress hormones raise glucose directly, which is why readings can climb on a difficult day even when you have eaten well. Infections and steroids do the same. Keep hydrated, keep testing more often when unwell, and do not stop prescribed medication without advice.

Alcohol

Alcohol can cause delayed low blood sugar, especially with insulin or sulfonylureas. If you drink, keep it moderate, never on an empty stomach, and check your glucose before bed.

5. Monitoring and troubleshooting

Use readings as information, not a verdict

A single high number is data, not failure. Look for repeating patterns over a week — that is what you can actually act on. Pair readings with a short note about the meal, activity, sleep and stress so the cause becomes visible.

Pattern	Common causes	Things to try
High in the morning	Dawn hormone rise, late heavy dinner, too little basal medicine	Earlier, lighter dinner; a short evening walk; review medicine timing with your prescriber
High 2 hours after meals	Large or refined carbohydrate portion, low fibre, drink with sugar	Cut the starch portion by a third; add protein and vegetables first; walk afterwards
Low readings (hypos)	Skipped or delayed meals, extra activity, alcohol, medicine dose	Treat immediately with 15 g fast carbohydrate, recheck in 15 minutes, then tell your prescriber
Swinging highs and lows	Irregular meal times, grazing, over-correcting	Fix meal timing first, keep portions consistent day to day, avoid stacking corrections

Know the hypo rule of 15

If your glucose is below 70 mg/dL (3.9 mmol/L): take 15 g of fast-acting carbohydrate (glucose tablets, half a cup of juice or regular soft drink), wait 15 minutes, recheck, and repeat if still low. Follow with a small snack if your next meal is more than an hour away.

When to contact your clinician

- Repeated readings well above or below your agreed targets.
- Any severe hypo needing someone else's help, or hypos you cannot feel coming.
- Illness with vomiting, or high readings with ketones.
- New numbness, tingling, vision changes, non-healing wounds or unexplained weight loss.

Important note

This guide is general nutrition education only. It is not medical advice, a diagnosis, or a treatment plan, and it cannot account for your personal history, laboratory results, allergies or medicines. Always speak to your doctor, dietitian or pharmacist before making significant changes to your diet, supplements or activity — especially if you are pregnant or breastfeeding, have kidney, liver or heart disease, an eating disorder, or take prescription medication. If you experience severe or sudden symptoms, seek urgent medical care. Diabetes medicines such as insulin and sulfonylureas can cause low blood sugar if your carbohydrate intake changes; doses may need adjusting by your prescriber.

Questions or want a personalised plan?

This guide covers general principles. A one-to-one consultation with The Nutrition Club Clinic lets us tailor portions, meal timing, food preferences, budget and any medications or conditions to you.

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